

Lab Serial No.	: 251909001490	Category	: BTC
Patient Name	: Mr. MANGERAM DHANKHAR	Reg. Date	: 16-Sep-19 09:50 AM
Age/Sex	: 64 YRS / M	Sample.coll.Date	: 16-Sep-19 10:01AM
Referred By	: Dr. SELF	Report Date	: 16-Sep-2019 03:33PM
TestName	: CBC		
Center	: HOME COLLECTION		

Test Name	Observed Value	Unit	Biological Ref Interval
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HAEMATOLOGY

CBC

TOTAL LEUCOCYTE COUNT (WBC)	14.6	X 10 ³ / μ L	4.0-10.0
DIFFERENTIAL LEUCOCYTE COUNT			
NEUTROPHILS	81	%	40-80
LYMPHOCYTES	12	%	20-40
MONOCYTES	05	%	2-10
EOSINOPHILS	02	%	1-6
BASOPHILS	00	%	0-1
ABSOLUTE NEUTROPHIL COUNT	11.83	X 10 ³ / μ L	2.0-7.5
ABSOLUTE LYMPHOCYTE COUNT	1.75	X10 ³ / μ L	1.0-4.0
ABSOLUTE MONOCYTE COUNT	0.73	X 10 ³ / μ L	0.2-1.0
ABSOLUTE EOSINOPHIL COUNT	0.29	X 10 ³ / μ L	0.02-0.5
ABSOLUTE BASOPHIL COUNT	0	X10 ³ / μ L	0.00-0.30
TOTAL RBC	3.69	million/ μ L	4.5-5.5
HEMOGLOBIN	9.7	gm/dl	13.0-17.0
PLATELET COUNT	206	x1000/mcL	150-410
HEMATOCRIT	30.0	%	40-50
MEAN CORPUSCULAR VOLUME (MCV)	81.3	fL	83-101
MEAN CORP. HEMOGLOBIN (MCH)	26.2	pg	27-32
MCH CONCENTRATION (MCHC)	32.2	g/dl	31.5-34.5
RED CELL DIST. WIDTH (RDW-CV)	17.9	%	11.5-14.5
RED CELL DIST. WIDTH (RDW-SD)	52.1	fL	39 - 46

Sample Type: EDTA Whole Blood Sample

Technology: Fully Automated Haematology Analyzer: Beckman Coulter Unicel DxH 800

Methods: Total Leucocyte Count: Coulter Principle; Differential Leucocyte Counts: VCS/Microscopy; Hemoglobin: Modified non-cyanmeth Method; Platelet Count: Coulter Principle, Total RBC: Coulter Principle; Haematocrit, MCV, MCH, MCHC: Calculated

Sample Type: EDTA Whole Blood Sample

Technology: Fully Automated Haematology Analyzer: SYSMEX XN-550

Methods: Total Leucocyte Count: Flow Cytometry; Differential Leucocyte Counts: Fluorescence Flow Cytometry/Microscopy; Hemoglobin: Cyanide Free SLS Method; Platelet Count: Electrical Impedance, Total RBC: Impedance Method with Hydrodynamic Focusing; Haematocrit, MCV, MCH, MCHC: Calculated

Remarks: Please correlate with clinical conditions.

Ruhani Kanwar

Dr. Ruhani Kanwar
Consultant Pathologist
M.B.B.S., M.D. (Pathology)
DMC Reg. No.: 88891

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Referred By : Dr. SELF	Report Date : 16-Sep-2019 02:32PM
TestName : TYPHI DOT (IgG + IgM)	
Center : HOME COLLECTION	

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SEROLOGY

DENGUE NS1

DENGUE NS1 NEGATIVE

Advise: Please note that this is a screening test only. Advised confirmation with alternative methods like RT-PCR or Dengue IgM ELISA.

Sample Type: Serum

Method: Rapid Test is a lateral flow Immunochromatographic assay.

Remarks: Please correlate results clinically.

MALARIA ANTIGEN

PLASMODIUM FALCIPARUM	Negative	Negative
PLASMODIUM VIVAX	Negative	Negative

Clinical Notes:

1. This test is an immunochromatographic assay for the differential detection between Plasmodium falciparum Histidine-Rich Protein-II (HRP-II) and pLDH (Plasmodium lactate dehydrogenase) specific to Plasmodium vivax in human whole blood.
2. This test should be used only for an initial screening and reactive specimens should be confirmed by a supplemental assay such as microscopic examination of thin blood smear.
3. The test must be correlated with other clinical data available to the physician.
4. In the gametogenic stage, P.falciparum may not be secreted. Such carriers may show a falsely negative results.
5. This test may be used to indicate a therapeutic response. Positive test results 5-10 days post-treatment indicate the possibility of a resistant strain of malaria.

Sample Type: EDTA

Method: Rapid Diagnostic Test

Please correlate results clinically.

TYPHI DOT (IgG + IgM)

TYPHI DOT (IgG)	POSITIVE
TYPHI DOT (IGM)	POSITIVE

Clinical Significance :-

- (1) TyphiDot (IgG) is indicative of old infection.
- (2) TyphiDot (IgM) measures recent infection.
- (3) These tests are intended to be used as screening test. Positive test must be Confirmed with confirmatory testing method(s).
- (4) Specimens containing unusually high titre of heterophile antibodies or RA factor may give false positive results.
- (5) A negative result indicates absence of detectable antibodies. However, a negative test does not rule out S.Typhi.

Sample Type: Serum

Test Method: Rapid Card Method

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Please correlate results clinically.

*** End Of Report ***

In case of any discrepancy, please contact the laboratory immediately. Since there is limited clinical interaction with the patient/physician, the report is not valid medico-legally.



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